

Healthcare Equity Beliefs and Moral Judgements of Patients

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Introduction

Healthcare equity is a fundamental principle of ethical medical care. The CDC defines health equity as “the state in which everyone has a fair and just opportunity to attain their highest level of health.” (CDC, 2024). Many believe that all people should have equal access to quality treatment, regardless of their circumstances. However, attitudes about healthcare sometimes become complicated when patients are perceived as personally responsible for the condition of their health. Individuals who struggle with lung issues associated with smoking and other lifestyle-related conditions are frequently judged more harshly and viewed as less deserving of medical care, compared to patients who deal with illnesses understood to be outside of their control, like a genetic disease. One 2020 study aiming to understand smoking-related stigma concluded with a recommendation to clinical nurse specialists, claiming that they “should promote anti-smoking campaigns that are not stigmatizing” and “discuss health risks of smoking in a respectful manner” (Draucker et al., 2020). Judgements of people’s health conditions, in both healthcare and everyday contexts, often overlook the

influence of the social determinants of health, like socioeconomic status, education, systemic inequality, and built environment, which strongly shape health behaviors and are highly predictive of many health outcomes. This raises the following question: Do beliefs about healthcare equity influence how individuals morally judge patients with diverse backgrounds and differing levels of perceived personal responsibility?

Background

One perspective on why people seem to support healthcare equity in principle while still judging certain patients more harshly comes from theories of moral intuition. According to Jonathan Haidt’s social intuitionist model, “moral intuitions come first and directly cause moral judgements”, referring to them as quick, automatic, and “a kind of cognition, but not a kind of reasoning” (Haidt, 2001). This theory suggests that reactions toward patients perceived to be responsible for their health issues may be influenced by this deeply moral and emotional automatic response, working alongside held beliefs about equity and fairness.

Existing literature has also shown that perceived personal responsibility has the

potential to influence attitudes in healthcare scenarios, with one 2006 national survey finding that “53% of Americans think it is “fair” to ask people with unhealthy lifestyles to pay higher insurance premiums and higher deductibles or copayments for their medical care than people with healthy lifestyles” (Steinbrook, 2006). Even global public health groups like the WHO are struggling to distinguish an out-of-control health risk from personal lifestyle choices as they advise entire nations, speaking on “a tobacco-related “epidemic” as if cigarette use were a contagious disease like malaria.” (Wikler, 2002). This suggests that something deeper is interfering with implementing equitable healthcare.

Finally, much of the current literature examining healthcare inequity emphasizes the importance of social determinants of health, including socioeconomic status, education, and built environment, which shape health outcomes far beyond an individual’s choice. Certain groups of people are adversely affected by health disparities and Healthy People initiatives have been attempting to tackle this systemic issue for over two decades. Policies have been implemented, trying to “attain the highest level of care among all population groups across America (ie, health equity).” (Williams et al., 2016). Yet, certain chronic conditions remain most prevalent in disadvantaged populations, so a divide is still clear, calling for new advancements. This study aims to

approach healthcare disparities from an interdisciplinary perspective by investigating the intuitive moral judgements that precede perceptions of personal responsibility, looking closer at how these possible biases might be contributing to inequitable healthcare attitudes and treatment.

Study Proposal

Hypothesis

This study proposes that individuals who endorse beliefs related to healthcare equity will show smaller differences in their judgements of patients with varying levels of assumed personal responsibility for their health issues. More specifically, participants who show strong beliefs in healthcare equity are expected to show similar levels of sympathy, blame, and deservingness towards patients whose illnesses are viewed either as self-caused or outside of their control. Participants with weaker beliefs in healthcare equity are expected to show greater differences in these judgements of perceived responsibility.

Participants

Participants will be made up of a diverse group of adults recruited through an online research platform. Participants coming from varying demographics like age, gender, education, and profession, may increase the generalizability of findings.

Procedure

First, participants will complete a questionnaire to assess their beliefs about healthcare equity and personal responsibility for health outcomes. The questionnaire will contain 15 statements rated on a five-point Likert scale. Statements will measure how strongly participants endorse healthcare equity, their attitudes toward unhealthy behaviors, and their beliefs about how certain health circumstances should be treated. Example statements may include: "Great healthcare should be provided to all people equally, no matter how well they take care of their body." and "People who engage in unhealthy behaviors should feel fully responsible for their negative health outcomes.". These sample items will measure how strongly participants support healthcare equity, no matter the patient's situation, along with the level of importance they place on accountability.

After completing the questionnaire, participants will read five short passages, each about a patient scenario that vary in levels of personal responsibility for the health issue. Each scenario will include contextual background information that may influence the participant's moral judgement. Examples of this may include socioeconomic background, education level, and access to resources. An example passage may include: "Patient is a 27-year-old male with severe opioid withdrawal symptoms who has dealt with

addiction ever since his back surgery two years ago. He was not aware that the painkiller prescribed to manage post-operative pain had such addictive properties. He has found the drug illegally to keep up with his dependence. He is now experiencing extreme vomiting, muscle pain, and anxiety, and is looking to be treated immediately for his symptoms.". Other scenarios will present health issues and illnesses viewed as either largely uncontrollable or very preventable.

After being presented with each scenario, participants will provide a rating for the patient on measures of sympathy, blame, and deservingness of medical treatment, again using a five-point Likert scale. These ratings will measure whether stronger beliefs supporting healthcare equity actually reduce judgement differences across patient scenarios with variance in perceived personal responsibility.

Measures

The primary independent variables in this study are the participants beliefs about healthcare equity and personal responsibility for health outcomes. The dependent variables measured are the participants ratings of sympathy, blame, and deservingness of treatment for their issues. Sympathy intends to measure the emotional concern present for the patient, blame assessing the extent to which they believe the patient should be held responsible for their state, and

deservingness giving information on beliefs of whether the patient should receive prompt, quality medical care.

Predictions

It is predicted that participants who strongly endorse healthcare equity will show smaller differences in their judgements across patient scenarios with varying levels of perceived personal responsibility. It is expected that these participants will provide similar ratings of sympathy, blame, and deservingness, regardless of how much control the patient had over their health condition. Participants with weaker beliefs in healthcare equity are expected to show a greater difference in their judgements, particularly by assigning more blame and less deservingness of quality healthcare to the patients they assume to be responsible for their condition. It is also expected that a certain degree of bias will remain present during these judgements, even among those who show strong support for healthcare equity. This suggests that moral intuitions and judgements may be influencing attitudes toward patients in healthcare settings, preventing the proper care that all utilizing healthcare services deserve.

Implications

This study may provide insight into the relationship between stated and deeply held beliefs about healthcare equity and moral judgement in healthcare contexts. If the hypothesis is well supported, the

findings would suggest that upholding beliefs supporting and endorsing healthcare equity may reduce biased judgements leading to proper treatment of patients, regardless of their responsibility for their condition. This would highlight both the difficulty and necessity of keeping moral evaluations separate from treatment plans. This study would contribute valuable support for healthcare professionals working towards a more equitable healthcare environment, directly demonstrating how their own attitudes are likely to shape the care they provide. Understanding this kind of bias may play a practical role in dealing with patient cases involving stigmatized conditions like lifestyle-related illness and addiction.

Additionally, this study may contribute to discussions highlighting the social determinants of health, which recognize that health outcomes are strongly correlated with many social factors, especially systemic inequality. Patients are often judged as personally negligent of their health without the full consideration of broader factors that may be limiting their ability to lead their healthiest life. The inaugural constitution of the WHO states that “the enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being without distinction of race, religion, political belief, economic, or social condition.”, yet it has nearly been a century since this was written and “it is arguable how much progress society has

made in achieving health equity, particularly in the US” (Chen, 2025). This study has the potential to see moral evaluations unintentionally reinforcing healthcare inequities we see all over the US, by viewing healthcare systems from a different angle. These findings may encourage healthcare professionals to closely monitor the impact of external social factors on each patient’s health status. Finally, this study may encourage future research that explores new methods which work to reduce the influence of moral judgements in healthcare.

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